**جوستاف روسي انترناشيونال - مصر (EPE) *** **طلب علاج أول مرة / متردد**

**إسم المريض رباعياً :** سلوي رزق مسعد جرجس
**المحافظة :** الجيزه **الحي / المدينة :** بولاق الدكرور **العنوان :** 0
**رقم البطاقة :** 26509120103642 **رقم التليفون :** 01222190662 **تاريخ تسجيل الإستمارة :** 24-10-2026
**العمر(تاريخ الميلاد) :** 12-09-1965 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

Name: سلوي رزق مسعد جرجس
National ID: 26509120103642

## تقرير طبي

السيد

تحية طيبة وبعد,,

| | |
|---|---|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم: | رقم طبي: |

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

| | |
|---|---|
| تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd | تاريخ وساعة الخروج : PM 01:17 05-07-2026 |

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

| | |
|---|---|
| التاريخ : 05-07-2026 | الوقت : -- --:-- |
| | الطبيب المعالج : د/شيماء لاشين |



سلوى
رزق مسعد جرجس
٤٠ ش سلامه عيد-اميشه
بولاق الدكرور - الجيزه

٢٦٥٠٩١٢٠١٠٣٦٤٢

١٩٦٥/٠٩

LF2682507

٢٦٥٠٩١٢٠١٠٣٦٤٢ ٢٠٢٥/٠٦
حاصلة على بكالوريوس تجارة

انثى مسيحية متزوجة
جرجس اسكندر عبدالمسيح روفائيل
البطاقة سارية حتى ٢٠٣٢/٠٦/٢٥ ١

| | |
|---|---|
| **Patient's Name:** | **Salwa Rizk Mosaad** |
| **Date:** | **2026-08-09** |
| **Referred by:** | Prof.Dr. **no doctor** |
| **Hospital:** | **Goustave Roussey International Egypt** |



**MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-**

- An enhancing neoplastic mass lesion is seen in the upper outer quadrant ,periareolar region of the right breast and measures about 32x23mm in dimensions(versus 32x23mm in the previous study) with irregular outlines and few foci of calcifications scattering inside. The overlying skin is thickened,more in the peri-areolar region and measures about 7.8 mm.
- **Normal left breast.**
- **Bilateral benign -looking axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 18 mm in the right side.**
- Hyperinflated both lungs.
- A newly-developed patchy area of consolidation is seen in the posterior segment of right upper pulmonary lobe.
- Linear atelectatic bands are seen in the middle lobe,lingula and in both lower pulmonary lobes with few bulla noted in both lower pulmonary lobes and the largest one seen in the left side and measures about 40x19mm in dimensions.
- Newly-developed enhancing pretracheal lymph nodes are seen and globally measure about 23x15mm in dimensions.
- No sizable abnormally-enhancing pulmonary nodules ,other patchy areas of consolidations or foci of calcifications noted.
- No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardiac effusions seen.
- Average size of the heart with normal CT of its big vessels.
- **Increased dorsal kyphosis** with foci of osseous sclerosis scattering in the scanned spine,more in S1,**diffuse osteoporosis,spondylo-degenerative changes of the scanned spine and degeneration of of multiple dorsal discs.**
- Mild increased size of the liver, measuring about 16cm , showing mildly diffuse reduced parenchymal attenuation and a homogenous enhancement pattern with persistence of the area of fatty spare in its left lobe.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.

CT Scan
[[DOCTYPE:CT_SCAN]]

| | |
|---|---|
| **Name:** سلوي رزق مسعد | |
| **MR. :** 622 | |

Detailed Medical Report

**السيد**

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | | | |
|---|---|---|---|
| **تاريخ وساعة الدخول :** | | **تاريخ وساعة الخروج :** | |

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**The patient is a postmenopausal individual with right-sided breast cancer, estrogen-receptor (ER) score 6/8, progesterone-receptor (PR) score 6/8, HER2 3+ amplified, presenting with metastatic disease to the liver (CT2) and bone (ER6/8 PR6/8 HER2+). Initial systemic therapy was planned with weekly taxol for 12 weeks and trastuzumab (herceptin) beginning in December 2025.**

**The disease was restaged as postmenopausal right breast cancer, clinical stage T3 N2 M1 invasive ductal carcinoma, ER 6/8, PR 6/8, HER2-positive by SISH amplification, with bone metastases and benign hepatic lesions. The treatment plan included taxol 140 mg weekly (starting week 12) and trastuzumab 440 mg IV (starting cycle 6), with an ejection fraction of 67 % on 6 January 2026; follow-up visits were scheduled for weeks 15, 16, and 17.**

**At a weight of 75 kg, height 150 cm (BSA 1.76 m²), the patient received trastuzumab 440 mg IV (cycle 7) and taxol 140 mg weekly (week 18). Plans were made to shift to a higher-dose schedule in weeks 19–20 pending pharmacy coordination.**

**The multidisciplinary team (MDT) reviewed metastatic bone disease on 23 February 2023 and recommended continuation of taxol and trastuzumab for two additional months, followed by a switch to trastuzumab plus an aromatase inhibitor if response persisted. The patient presented for week 21 taxol 140 mg and cycle 8 trastuzumab 440 mg IV; lower-extremity edema with cellulitis-type redness prompted a duplex study, leading to temporary hold of taxol. Laboratory work was pending for week 22, with taxol planned for week 23; the most recent echocardiogram on 6 January 2026 showed EF 67 %.**

**The patient attended for week 23 taxol after laboratory results were available and received cycle 9 trastuzumab 440 mg IV. Pharmacy prepared for week 24 taxol and week 25 taxol administration.**

**Pharmacy documentation confirmed the patient's weight (75 kg), height (150 cm), and BSA (1.76 m²) for week 24 taxol 140 mg, with labs from 29 March accepted.**

**A new CT scan was pending; clinical examination revealed no right-breast mass. MDT planning for week 26 included taxol 140 mg and cycle 10 trastuzumab 440 mg IV (EF 68 % on 5 April 2026) with grade 1 peripheral neuropathy noted.**

**Bone scan on 1 January 2026 showed benign fatty infiltration in the liver and a dorsal vertebral lesion (D9-D11, L2). The patient was scheduled for zoledronic acid (Zometa) and taxol in week 27, followed by pharmacy preparation for week 28.**

**Pharmacy recorded acceptance of laboratory results for week 28 taxol 140 mg on 18 April.**

**تحريرا في ,**

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| **التاريخ :** | **20-09-2026** | **الوقت :** | |

**الطبيب المعالج :** **dina makram**

**يعتمد ,**

**MDT in May 2026 discussed continuation of an aromatase inhibitor (arimidex) with trastuzumab, zoledronic acid, and evaluation every three months; cycle 11 trastuzumab 440 mg was planned, and the patient had already started arimidex.**

**On 19 May, the patient was prescribed arimidex and supportive medication for the next two months; examination revealed bilateral lower-extremity edema, and full laboratory work was ordered.**

**For cycle 12, trastuzumab 440 mg IV was administered together with ongoing arimidex; a lower-extremity venous duplex study was ordered.**

**Cycle 13 consisted of trastuzumab 440 mg IV, continued arimidex, and supportive therapy, with a repeat echocardiogram scheduled before the next cycle.**

**During the visit on 12 July, the patient received cycle 14 trastuzumab 440 mg IV and continued arimidex with supportive care; echocardiogram on 5 July showed EF 66 %. New CT imaging demonstrated progression of cancer in the pancreas (CA P) and bone involvement, prompting a recommendation to proceed with cycle 15 trastuzumab 440 mg, arimidex, and supportive therapy, with pharmacy preparation for cycle 16.**

**The most recent note on 2 August documents new CT evidence of pancreatic cancer and bone metastases, and the plan to administer cycle 15 trastuzumab 440 mg, continue arimidex, and provide supportive treatment, with cycle 16 being prepared in pharmacy. The patient has completed 15 cycles of trastuzumab to date.**

جوستاف روسي انترناشونال

| | |
|---|---|
| **Patient's Name:** | **Salwa Rizk Mosaad** |
| **Date:** | **2026-08-09** |
| **Referred by:** | Prof.Dr. **no doctor** |
| **Hospital:** | **Goustave Roussey International Egypt** |

- Average sized spleen,measuring about 9.9 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- A right ovarian cyst is seen and measures about 33x23mm in dimensions.
- The rest of the pelvic structures are unremarkable.

**IMPRESSIONS:**

**A status follow up,and compared with the previous study,the current examination revealed:**

- **An enhancing neoplastic mass lesion in the upper outer quadrant ,periareolar region of the right breast and having the above-mentioned criteria and extension with a stable course.**
- **Hyperinflated both lungs with a newly-developed patchy area of consolidation in the posterior segment of right upper pulmonary lobe and persistence of the linear atelectatic bands in the middle lobe,lingula and in both lower pulmonary lobes with few bulla noted in both lower pulmonary lobes as well.**
- **Newly-developed enhancing pretracheal lymphadenopathy with foci of osseous sclerosis scattering in the scanned spine,more in S1.**
- **Mild hepatomegaly with mild fatty infiltration and no focal lesions.**
- **A small right ovarian cyst.**

**Prof/Hanan El-Ahwal.**

CT Scan
[[DOCTYPE:CT_SCAN]]

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|---|---|
| **Patient's Name:** | **salwa rezk mossad gerguis** |
| **Date:** | **2026-04-18** |
| **Referred by:** | Prof.Dr. **ct cap iv** |
| **Hospital:** | **Goustave Roussey International Egypt** |

***MULTI-DETECTOR C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS FOLLOWING ORAL AND INTRAVENOUS CONTRAST MEDIA ADMINISTRATION REVEALED:***

**Reported case of right breast carcinoma. Compared to the last study dated 20-12-2025 ,the current study revealed:**

- No appreciable time interval changes regarding the size of the previously seen right breast spiculated mass still measuring 3.2x 2.3 cm for sono-mammography
- Multiple bilateral scattered atelectatic bands.
- Normal lung parenchyma, showing no focal lesions.
- Normal vascular mediastinal structures.
- The trachea, main stem bronchi as well as their lobar and segmental divisions are normal.
- No evidence of mediastinal or hilar nodal enlargement.
- No pleural collections.
- The liver is still seen enlarged in size showing homogenous reduced CT density with rather stationry course regarding the previously seen segment IV hypodense area around the bifurcation of the portal vein, measuring 34x28x26 mm in diameter with no pathological enhancement in the different phases of imaging…likely **focal fatty infiltration.**
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Unremarkable pelvic organ.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Scanned lung bases are clear.
- Still noted dorsolumbar spondylotic changes.

**OPINION:**

**Stationary course since last study.**

***Dr.sherif Wagdy***

CT Scan
[[DOCTYPE:CT_SCAN]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

02/23/2026

# MDT FORM

| | | | |
|---|---|---|---|
| **Name:** | Salwa Rizk Mosad . | **MRN:** | 622 |
| **Age:** | 60 Y | **ID:** | 2290-2025 |
| **Menopausal Sta** | | **Phone:** | 01212890854 |
| **NRS SCORE:** | | **Marital Status:** | |
| **BMI:** | | **Family History:** | |
| **CO Morbidity:** | | **Reason Of MDT:** | |

## Clinical Data:

**BX:** IDC2

**ER:** 6/8 **PR:** 6/8 **HER2:** AMPLIFIE **KI67:**

**Sonomammography:**

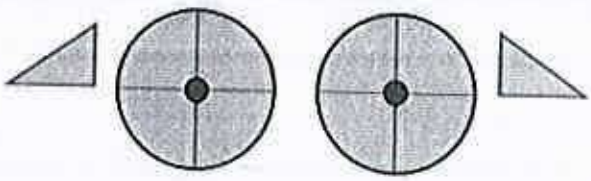
**Mets Workup:**
postm rt breast cancer ct3 n2 m1 met to bone rec taxol and herceptin cts 12/2025 sd bs 12/2025 sd pt rec 18 weeks taxol and 7 herceptin

**History:**

**Recommendations:**

**MDT Outcome & Plan**
if no PN keep on Taxol/ Herceptin for 2 more months then shift to AI + Herceptin if still responding.
if developed PN G3 shift now to AI + herceptin

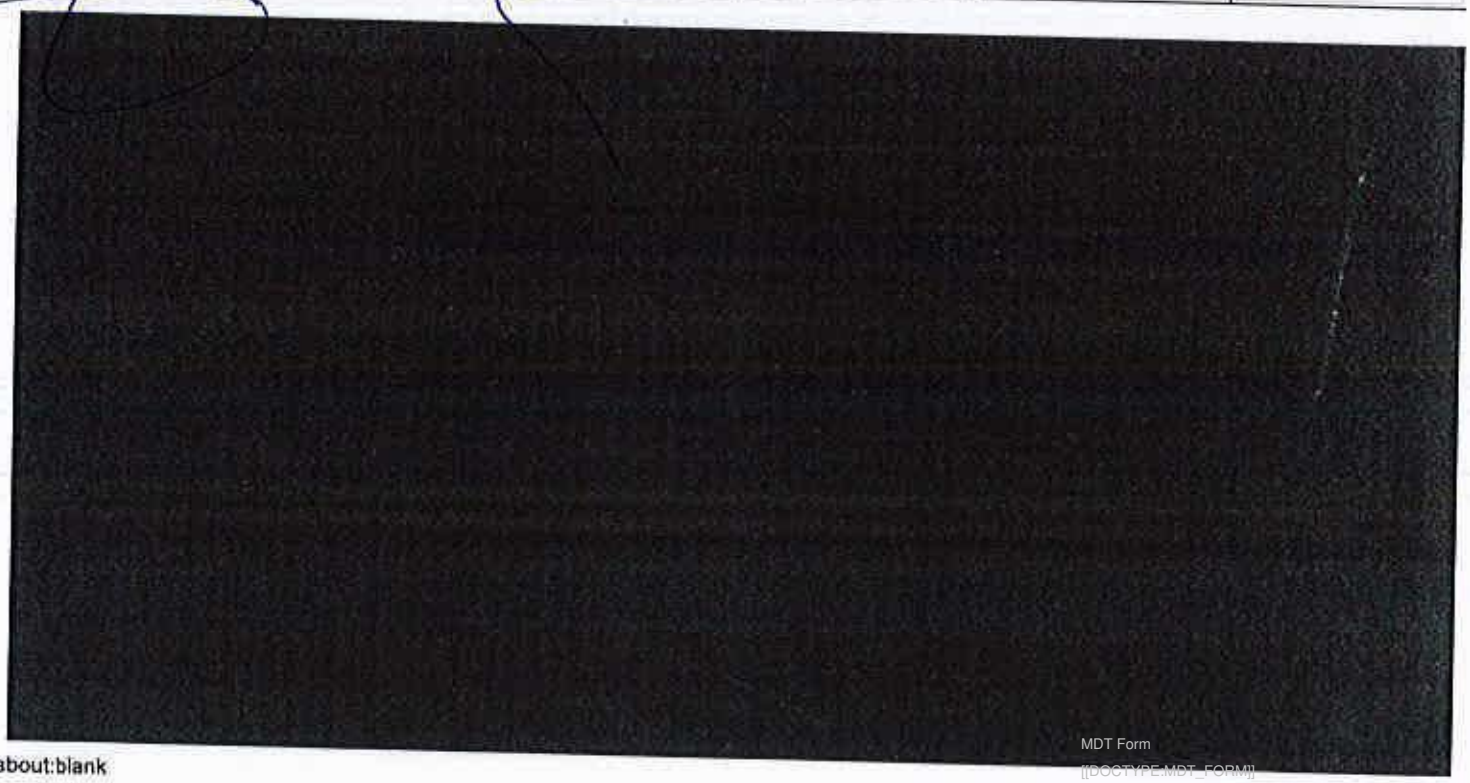
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**TNM**

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|---|---|---|---|---|---|
| | | | | | |



GUSTAVE
ROUSSY

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|---|---|---|---|
| Patient Name | : الأستاذه / سلوي رزق مسعد جرجس | Request No. | : 13009131 |
| Patient No. | : 26509120103642 | Request Date | : 31/07/2025 |
| Path No. | : SR-1054/25 | Verification Date | : 02/08/2025 |
| Sex | : Female | Financial Category | : جوستاف روسي - ثقة الدولة |
| Age | : 59 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical Data
Right breast mass

Nature of specimen
Block &slide revision

Gross examination
One block labelled by (25/173) &6 slides were received.

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
* DCIS:Absent
* Angioinvasion:Positive
*Perineural invasion: Absent
*Microcalcifications : Absent
* IPT revision :
* ER:Strong nuclear reaction in 20 % tumor cells (Allred score 6/8)
* PR: Strong nuclear reaction in 30 % tumor cells (Allred score 6/8)
* HER2: Moderate compelete membrane reaction in>10 % tumor cells (score 2+) Equivocal
*SISH : Amplified
-Average Her2 VCopy No :5.4
-HER2/CEP17 ratio :2.13
* KI 67:20 %

Diagnosis

Verified by :
Dr.Heba Hasan



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE/
OUSSY

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذة / ساوي رزق مسعد جرجس | Request No. | : 13009131 |
| Patient No. | : 26509120103642 | Request Date | : 31/07/2025 |
| Path No. | : SR-1054/25 | Verification Date | : 02/08/2025 |
| Sex | : Female | Financial Category | : جوستاف روسي - تلة الورلة |
| Age | : 59 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

Right breast mass, Block &slide revision:
Invasive duct carcinoma, Grade 2 .
IPT:

- ER: Positive (score 6/8).
- PR: Positive(score 6/8).
- HER2:Equivocal (score 2+).
- SISH : Amplified
- KI67: 20 %

- Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

- Scoring system: "Her2"
- Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Verified by :
Dr.Heba Hasan



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE/
OUSSY

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذ / ماري رزق مسعد جرجس | Request No. | : 13009131 |
| Patient No. | : 26509120103642 | Request Date | : 31/07/2025 |
| Path No. | : SR-1054/25 | Verification Date | : 02/08/2025 |
| Sex | : Female | Financial Category | : جوستاف روسي - نقابة الدولة |
| Age | : 59 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

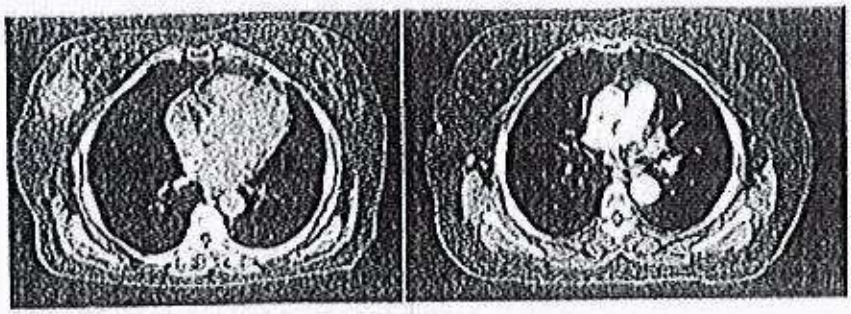
GUSTAVE ROUSSY
INTERNATIONAL MEDICAL

| | |
|---|---|
| Patient Name: Salwa Rezk Mossad Gerguis | Date : 24/08/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis) | Age : 60 Year(s) |
| Medical ID: 229/2025 | Pat. ID: 111095 |

## *POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -*

*Clinical History:*

- *Known case of right breast cancer, no available previous films for comparison.*



*Findings:*

- The right breast shows upper outer quadrant speculated soft tissue mass lesion measuring about 4x4 cm.
- Few subcentimetric right axillary lymph nodes are noted, the largest seen measuring 9x6 mm.
- Few subcentimetric left axillary lymph nodes are noted.
- Both lungs show scattered fibro-atelectatic bands.
- No sizable nodules, masses or consolidative patches.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

*OPINION:*

- *Right breast UOQ speculated soft tissue mass lesion for sonomammography correlation.*
- *Few subcentimetric right axillary lymph nodes.*
- *Few subcentimetric left axillary lymph nodes.*

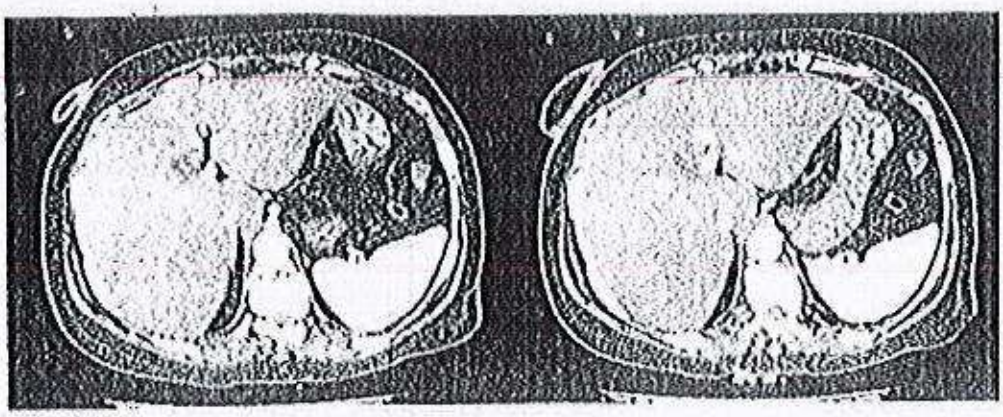
*Marina Essam, MSc*
*Dr.Noura Nabil,MD*

## *MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS: -*

*Clinical History:*

- *Known case of right breast cancer, no available previous films for comparison.*

CT Scan
[[DOCTYPE:CT_SCAN]]



*Findings:*

- The liver is enlarged (20.5 cm) with diffuse low homogenous CT attenuation showing ill-defined geographical segment IV hypodense areas (?? Areas of fatty infiltration/sparing) for triphasic CT correlation.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size of the spleen (12 cm) with no focal lesions seen.
- Normal appearance of the pancreas and both suprarenal glands.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Average filling of the urinary bladder with no sizable stones or masses.
- Advanced spondylo-degenerative changes are noted.

*Opinion:*

- ***Fatty hepatomegaly with few ill-defined geographical segment IV hypodense areas (?? Areas of fatty infiltration/sparing) for triphasic CT correlation.***

*Marina Essam, MSc*
*Dr.Noura Nabil,MD*

Signature Noura Nabil
28/08/2025 1:16 AM

CT Scan
[[DOCTYPE CT_SCAN]]

Name : Salwa Rizk Mosaad Girgis
Branch : Dokki

Date : 05-09-2025 09:47
Medical No. : 195020

# $Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

**History:**

- 60-year-old lady, with right breast cancer, referred for metastatic workup.

**Technique:**

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

**Findings:**

- Foci of enhanced tracer uptake are seen at DV9, DV10, DV11 and LV2.
- No other abnormal active or cold focal bony lesions could be seen allover the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

**Impression:**

- Few active vertebral lesions; likely metastatic, for correlation with radiological data and follow up.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

[[DOCTYPE:BONE_SCAN]]
Bone Scan



Name : Salwa Rizk Mosaad Girgis
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt
Date : 31-07-2026 05:33
Medical No. : 291739
Ins.Company : تعاقد مستشفيات

# Tc-99m MDP Skeletal Scintigraphy

History: 60-year-old female with history of right breast cancer on hormonal therapy.

Technique: Anterior and posterior whole-body scans were acquired 2 hours post-injection of the tracer.

Findings:

- Compared to prior studies dated September 5, 2025, and January 10, 2026, persistent low-grade focal tracer uptake remains localized to the lower dorsal vertebrae (DV9, DV10, DV11) and upper lumbar vertebra (LV2), showing a stationary course with no significant interval progression.
- No other abnormal active or cold focal bony lesions are seen over the surveyed skeleton to suggest new metastatic bone disease.
- Mild degenerative changes are noted at the spine and major joints.
- Symmetrical renal uptake with physiological urinary excretion is observed.

Impression:

- Stationary course of the low-grade few vertebral lesions (DV9-DV11, LV2). Radiological correlation is advised.
- No new lesions depicted. Clinical correlation is advised.

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